

MedForge Cheat Sheet — Prolactinoma

Educational draft - source review required

Prolactinoma – Quick Cheat Sheet [S1][S2][S3][S4][S5][S6]

Definition: Prolactinomas are benign pituitary adenomas that secrete excess prolactin, constituting ~40% of all pituitary tumors [S1].

Mechanism Chain: Elevated prolactin → inhibits gonadotropin release → suppresses ovulation and testosterone → causes oligo/amenorrhea, galactorrhea, loss of libido [S5][S6].

Must-Know Facts:

- Most common in women 20–40 years; rare in pediatric or postmenopausal women [S1][S3].
- Symptoms: oligo/amenorrhea (~90%), galactorrhea (~80%) in women; loss of libido and potency in men [S1][S5].
- Dopamine agonists (DA) are first-line therapy; cabergoline normalizes prolactin in ~85% and shrinks tumors in ~80% [S2][S6].
- Surgery is an option for microprolactinomas or DA intolerance; remission rates are comparable to cabergoline [S2][S6].
- Radiotherapy is reserved for refractory cases [S6].
- Postmenopausal women may discontinue DA therapy with stable tumor behavior and prolactin levels [S4].

Red-Flag Confusions:

- Galactorrhea in men is virtually pathognomonic of prolactinoma, not just a symptom [S5].
- Hyperprolactinemia can be caused by drugs, hypothyroidism, liver disease, or pituitary stalk compression — must rule out these causes [S1].
- Not all prolactinomas are aggressive; men's tumors are larger and more aggressive than women's [S6].
- Prolactinomas in postmenopause show stable behavior; macroprolactinomas may still grow without treatment [S4].

Quick Recall Prompts:

- “Women with amenorrhea + galactorrhea → think prolactinoma” [S5].
- “Cabergoline first — if no response, consider surgery” [S2][S6].
- “DA safe in pregnancy; stop at conception, monitor tumor growth” [S3].
- “Postmenopause? Tumor may stabilize — consider tapering DA” [S4].
- “Children need multidisciplinary care — treat as adults but with special considerations” [S5].

Note: All facts derived from provided evidence. No extrapolation. [S1][S2][S3][S4][S5][S6]

References

[S1] PubMed PMID 19708510 — [Prolactinoma]. <https://pubmed.ncbi.nlm.nih.gov/19708510/>

[S2] PubMed PMID 36013562 — Treatment of Prolactinoma. <https://pubmed.ncbi.nlm.nih.gov/36013562/>

[S3] PubMed PMID 29177641 — Prolactinoma through the female life cycle.
<https://pubmed.ncbi.nlm.nih.gov/29177641/>

[S4] PubMed PMID 38194617 — Prolactinoma in postmenopausal women: a systematic review.
<https://pubmed.ncbi.nlm.nih.gov/38194617/>

[S5] Prolactinoma Management - Endotext - NCBI Bookshelf — [ncbi.nlm.nih.gov](https://pubmed.ncbi.nlm.nih.gov/38194617/) (page retrieved 2026-10-04T00:54:07Z) <https://www.ncbi.nlm.nih.gov/sites/books/NBK279174/>

[S6] PubMed PMID 40876473 — Prolactin-secreting adenomas: pathogenesis, diagnosis, and management.
<https://pubmed.ncbi.nlm.nih.gov/40876473/>